# ASTRAZENECA GLOBAL STANDARD OPERATING PROCEDURE

# MANAGEMENT OF IMPORTANT NEW SAFETY ISSUES

## 1. PURPOSE

The purpose of this SOP is to describe the process for identification, review and reporting of Important New Safety Issues, originating from any source, to Regulatory Authorities, Investigators and Ethics Committees (ECs) or Institutional Review Boards (IRBs).

## 2. AUDIENCE

This SOP applies to relevant employees (including contractors and consultants) of the AstraZeneca group of companies (AZ) engaged in the identification, review and submission of Important Safety Issues.

In addition, this SOP may be applicable to certain Third Parties and if so, this will be documented in the agreement/contract with the Third Party.

## 3. SCOPE

This SOP applies to activities for the following:

- Products for which AstraZeneca group of companies (AZ) has submitted a Marketing Authorization/License approval anywhere in the world including Products of Local Opportunity (PoLO) (as locally applicable).
- Any Approved Product or Investigational Medicinal Product (IMP) for which a clinical study sponsored by AZ is submitted to and/or approved by one or more Regulatory authorities.
  - **Note: Within this document, the term 'product(s)' includes AZ manufactured medical devices or a drug/device combination.**

The following are out of scope for this SOP and are described in other procedural documents, see:

- Routine reporting of Individual Case Safety Reports (ICSR) to Regulatory Authorities and/or EC/IRB and Investigators (Refer to SOP: Expedited Reporting of Individual Case Safety Reports to Regulatory Authorities) and SOP: Expedited Reporting of Individual Case Safety Reports to Investigators, Ethics Committees and Institutional Review Board). However, the reporting of ICSRs as an Unanticipated Problem is in scope.
- Crisis management. Refer to: AZ Global Crisis Management Standard and Crisis Toolkit.
- Issues which may have an impact for patients but do not fit the definition of a 'safety issue' for this SOP, including:
  - Issues impacting commercially distributed products for which Operations have oversight authority. Refer to: 7-P10-cv-X Management of Product

Quality Issues Affecting Distributed Product that may require In-Market Action.

- Issues impacting AZ IMPs for which Pharmaceutical Development have oversight authority. Refer to: 7-P11-cv-x Issue Management (Including Recall) of Investigational Medicinal Products.
- Issues relating to product security, complaints or suspect samples. Refer to:
  - 7-P34-cv-X Handling of Product Quality Complaints/Product Security with Associated Individual Case Safety Reports.
  - 7-P29-cv-X Global Complaint Management.
  - Handling of Suspect Samples. Refer to: QCM process map 1.12.2 Handling of Suspect Samples and Authentication Outcomes.

**Limitations:** This SOP does not apply to activities conducted before the effective date of this procedure.

For products that are co-developed (or co-marketed/co-licensed where applicable) with other business partners, AZ responsibilities for safety activities may be limited and will be specified in product, program or study specific contractual agreements.

# 4. PROCEDURE

## 4.1 General Principles

### 4.1.1 Important New Safety Issues

A safety issue arising from the identification of an unexpected and serious safety signal with a potential impact on the overall benefit/risk of the product or potential public health impact is referred to as an Important New Safety Issue (INSI). Due to the significant safety impact of these issues they require urgent attention and the potential need for prompt regulatory action and communication to patients and healthcare professionals. A safety issue may be described as new if it has not been previously reported/notified or is not a logical extension of knowledge or findings previously submitted to a Regulatory Authority (for example as part of the Market Product Information (MPI) or Investigator Brochure (IB) for that product).

If there is a doubt whether a safety issue meets the definition of an INSI this must be thoroughly assessed by the PS Safety Surveillance Team, Safety Strategy and Management Team (SSaMT) and Senior Pharmacovigilance Medical Director (SPVMD) to ensure alignment before being referred to the PS Therapeutic Area Vice President (PS TA VP) and QPPV for European Economic Area (EEA) products with marketing authorisation or application. See section 4.1.4 for dispute resolution.

An INSI will meet the criteria described below:

1. Is not already appropriately covered in the product’s core data sheet or IB

AND

2. Which affects the overall benefit/risk profile of the product, defined as

- For developmental products:
  - Necessitating a significant safety-related change in study conduct (i.e. recruitment/study progression, dosing or dose pause, inclusion/exclusion criteria, monitoring requirements, or ICF/IB update)
- For marketed products:
  - Sufficient to consider a change/restriction to the administration of a medicinal product either to all patients or to a specific subset of the population (e.g., elderly, organ-impaired, other at-risk groups)

AND

  - Where it is considered necessary by AstraZeneca to communicate the finding in an expedited manner to Health Authorities, Healthcare Professionals and patients to appropriately manage patient safety

#### 4.1.1.1 Important New Safety Issues During Non-clinical Studies

INSI may also originate from non-clinical studies. Similar to clinical INSI, these are safety issues arising from non-clinical studies that identify findings with a potential impact on overall benefit/risk of the compound or have a potential public health impact. Some examples include:

- Evidence of carcinogenicity, mutagenicity, teratogenicity or embryofetal death observed with compounds which are in active development (or actual use in patients) that are intended for use in diseases that are not life-threatening. For life-threatening diseases such evidence may still qualify as an INSI, based on the overall benefit risk profile.
- Functional, pharmacological or toxicological effects not previously seen in studies, that are considered severe enough to pose a risk to humans.
- An unexpected increase in the incidence or severity of a previously seen and medically important adverse effect or a new occurrence of a similar (medically important) adverse effect in a different species or at a lower exposure.
- An unexpected toxicity finding, an increase in the incidence or severity of a previously seen adverse effect, or new occurrence of similar effect when the drug is administered in combination or sequence with another agent

Lethality or organ toxicity, when identified in studies designed to provide such information, are not considered to be INSIs unless the potency of the compound indicates a high risk to humans in ongoing or previously conducted clinical trials.

#### 4.1.1.2 Issues with Potential Impact for Patients

Issues relating to the quality, supply or security of commercially available products or IMPs that do not fit the definition of ‘safety issues’ but require assessment by a qualified medical professional from within AZ, are referred to as ’issues with potential impact for patients’. The management of these issues is out of scope for this SOP (refer to procedures listed in Section 3).

There can also be issues when a safety signal is not observed but due to the severity of the issue expedited reporting is required. Some examples would include:

- Widespread breech of study participant or patient confidential health information
- For biologic products, study participants receive investigational product obtained from donors who were not appropriately screened and tested for several potential viral contaminants such as the human immunodeficiency virus or the hepatitis B virus
- Limitations or inability to supply product where such shortages impact patient safety or public health

Any such issue would be managed as an INSI per the processes outlined in sections 4.2 and 4.3.

#### 4.1.1.3 Important New Safety Issues Requiring an Issues Management Team

The identification and reporting of INSI may involve additional assessment and/or action associated with the safety issue. These issues would generally be of a critical nature and with a potential significant threat to patients, a product or the business as a whole. Such instances require the formation of an Issue Management Team to bring together all relevant functions and co-ordinate activities to resolve the issue. This would be proposed by the SSaMT.

### 4.1.2 Identification and Escalation of Potential Important New Safety Issues

#### 4.1.2.1 Non-clinical Sources

The Toxicology Project Lead (TPL) is accountable for identification and escalation of a potential INSI. The TPL will notify the SSaMT of any potential INSI and escalate to the Safety Assessment Review Board (SARB).

The SARB is the non-clinical safety body in Clinical Pharmacology and Safety Science (CPSS) with accountability to provide endorsement on matters to non-clinical safety within projects.

The SARB will review all available data and provide its recommendation if the safety issue meets the requirements of an INSI and, in SARB's view, should be expeditedly reported. If not considered an INSI, this decision will be communicated to the SSaMT and SPVMD. The SPVMD may disagree with this recommendation and escalate to PS TA VP and the QPPV (as appropriate, for products with an EEA marketing authorisation or application), see section 4.2.7. If expedited reporting is recommended, the TPL will notify the Global safety Physician (GSP) and SPVMD for the product. The SPVMD will escalate to the PS TA VP and EU QPPV. The PS TA VP makes the final determination if the INSI requires expedited reporting. See section 4.2 for details on timelines and process. See section 4.6 for documentation and retention of decisions.

#### 4.1.2.2 Clinical Development and Post-Marketed Sources

The GSP in consultation with the SSaMT and SPVMD is accountable for determining if a safety issue is a potential INSI and requires urgent escalation. If determined that escalation is required, the GSP will inform the SPVMD who

will present the safety issue to the QPPV (for products with an EEA marketing authorisation or application) and PS TA VP for endorsement of expedited reporting. The PS TA VP makes the final determination if the INSI requires expedited reporting. See section 4.3 for details on timelines and process. See section 4.6 for documentation and retention of decisions.

#### 4.1.3 Reporting of Important New Safety Issues and Assessment Limitations

INSIs require communication in an expedited manner to appropriately manage the safety of patients or clinical study subjects. As stated, this includes notification as an “Unanticipated Problem” or “Emerging Safety Issues” as defined under Food and Drug Administration (FDA) and European Medicines Agency (EMA) guidance respectively.

The requirement to expeditedly report INSIs within specific timelines (defined by regulatory requirements and International Standards) means that it may not be possible to conduct a complete assessment of causality or to fully determine further actions, such as label changes, before initial reporting. Initial submission of expedited reports must not be delayed. Any such limitations should be specified in the submitted report with, if appropriate, a commitment to provide further information or update at a later date.

Any issue management activities or Urgent Safety Measures (USM) (see Section 6 Glossary) associated with the INSIs must not delay reporting requirements as described in this SOP.

#### 4.1.4 Dispute Resolution

In the event of a dispute at any level regarding the decision to escalate a potential INSI, the matter must be referred by the SPVMD to the PS TA VP, and to the QPPV (for products with an EEA marketing authorisation or application) for a decision. If a decision cannot be reached the PS TA VP will escalate the issue to the Executive Safety Board (ESB) for final resolution. The QPPV also has single authority to escalate the matter to ESB (see QPPV SOP). The SSaMT members, SPVMD and TPL (for non-clinical INSIs) should be included in any discussion relating to the dispute.

Due to the potential impact on the overall benefit/risk of the product and public health, dispute resolution discussions must occur with the utmost urgency following initial awareness of the safety signal. If resolution cannot be reached within an appropriate timeframe and without undue delay, the default position is to proceed with reporting.

#### 4.1.5 Regulatory Clock Start Date

The regulatory clock start for reporting is the date the INSI has been endorsed by the PS TA VP (see Sections 4.2.8 and 4.3.6). This date should be considered day 0 (zero) for the purpose of calculating reporting timelines.

#### 4.1.6 Timing and Specific Requirements for Notifications to Health Authorities

In general, expedited reporting of INSIs is required “as soon as possible”, but in no case later than 15 calendar days from the Regulatory clock start date (day 0). However, time frames may be less than 15 calendar days as specified by requirements in different countries/Regulatory Authorities see Section 4.1.6.1.

For example, the UK Medicines and Healthcare Products Regulatory Agency (MHRA) must be informed immediately via telephone (end of calendar day 1).

It is the Marketing Company's (MC) responsibility to document their reporting requirement timeline for INSIs in the appropriate Global Regulatory Intelligence system, e.g. RADAR. This information will be used by Global Patient Safety and Regulatory teams to define reporting strategies for each country.

#### 4.1.6.1 European Economic Area (EEA) Authorized Products – Reporting Emerging Safety Issues

An INSI must be reported "as soon as possible" as an Emerging Safety Issue, per GVP IX this is within 3 working days. The report must be made in writing to the Regulatory Authorities in EEA Member States where the medicinal product is submitted/authorized.

The QPPV is responsible for reporting an INSI to EMA via email to:

P-PV-emerging-safety-issue@ema.europa.eu

Note that within the EEA, further instructions on the reporting process may be applicable according to the country/Regulatory Authority.

At a minimum the report must include:

- Description of the safety issue
- The sources of information pertaining to the INSI
- Any planned actions with timelines
- Provision of any relevant documentation available at the time of initial report

Any further information relevant to the issue should be provided to the EEA and relevant national competent authorities as soon as it becomes available.

If an action associated with an emerging safety issue results in temporary or permanent cessation or suspension of marketing of a medicinal product, withdrawal of a medical product or non-application for the renewal of a marketing authorization the notification of this action will be done in parallel to the EEA emerging safety issue notification via this e-mail: withdrawnproducts@ema.europa.eu

#### 4.1.6.2 FDA – Reporting Unanticipated Problems

Per the FDA, an unanticipated problem includes any incident, experience, or outcome that is:

- Unexpected
- Related or possibly related to participation in the research
- Places subjects or others at a greater risk of harm than was previously known or recognized.

Reports of Unanticipated Problems will include the same level of information outlined for reporting of Emerging Safety Issues (see 4.1.6.1). Safety issues that meet the Unanticipated Problem criteria require reporting to the FDA no later than 15 calendar days after it is decided that the safety issue requires expedited reporting. These submissions will be managed via the submission process described in Section 4.4.

### 4.1.7 Specific Considerations for Reporting

The development of INSI report content is a collaborative responsibility with members of the SSaMT. The SSaMTL has overall responsibility for the content of communications for a product and the Global Science Leader (GSL) and Clinical Delivery Lead (CDL) will author documents specific to clinical programs.

With the exception of reporting to the EMA, which is the responsibility of the EU QPPV, see section 4.1.6.1, Global Regulatory are responsible for submission of INSI reports to the impacted MC within required timelines (see section 4.1.6) and the MC are responsible for submission to their Health Authority. All submissions, both global and local, are tracked via a regulatory submission tracking system for both global to MC and MC to Health Authority submission.

For development and oversight of INSI reporting in studies to Regulatory Authorities, Study Investigators and Ethics Committees the GSL is accountable. The CDL is responsible for ensuring reporting to Regulatory Authorities, Study Investigators and Ethics Committees is performed and actions associated with the reports are documented within the study specific electronic Trial Master File (eTMF). The GSL is also responsible for ensuring the Investigational Product's Investigator Brochure is also updated accordingly.

The following points should be considered when planning the required submissions (See Section 4.4.2):

- Submissions requiring extensive translation into local language or requiring extensive supplementary information: In order not to delay reporting, a preliminary report must be submitted comprising the global package plus a cover letter in local language committing to providing the submission as per local Law. For example, this is applicable in Japan and the appropriate expert at AZKK should be contacted where reporting is required.
- Products used in combination: Consider whether the INSI applies to a single product and/or whether that product is also used in combination - either within clinical studies or submitted/approved products.
- Products used in External Sponsored Research (ESR): Consider whether the INSI applies to any products used by Externally Sponsored or Co-Sponsored studies and notify third parties as appropriate.
- Products included in more than one clinical study: Timely coordination is needed of reporting activities to Regulatory Authorities, ethics committees and Investigators across all impacted studies.
- Need for a Direct Health Care Professional Communication (DHPC): Determination whether a DHPC is required (see SOP 8-P130-CV-X Direct Health Care Professional Communication for DHPC (AZDoc0010091))

- Studies conducted under US Investigational New Drug (IND): For the US, the need for cross-reporting to multiple INDs must also be considered. This will be done if it is determined the INSI is applicable across multiple INDs.
- Past Study Patients*:* Relevant local MC should be contacted to identify if past study patients require notification in that country. For example, in Korea it is a requirement to inform relevant clinical study institutes of issues that may impact study subject safety or medical treatment decisions not only during the study but also for 2 years after completion of any study. The Korean MC Medical Director (or equivalent) must be informed of any INSI that require notification within this specified time period.
- IMPs under investigation in more than one indication, in which an INSI is identified in only one of these indications*:*
    - Investigators and Ethics Committees: Consideration will need to be given as to whether this will need to be communicated to all relevant Investigators and Ethics Committees. The default position will be that INSIs will be notified in an expedited manner to all relevant Investigators and Ethics Committees unless there is a clinical reason why expedited notification to certain groups of Investigators and Ethics Committees is not considered necessary. Where a clinical reason exists not to expedite a report to certain groups of Investigators and Ethics Committees, the GSL will be responsible for ensuring that this is rationale is provided to the SSaMTL and documented as described in Section 4.6.

Regulatory Authorities: Where the clinical studies for different indications are conducted in separate countries, it may only be necessary to submit an expedited report to the corresponding Regulatory Authorities. This must be agreed between the CDL and GRL and endorsed by the PS TA VP and QPPV (for EEA submitted/authorized Products only) and documented as described in Section 4.6. If an expedited report is not sent to all Regulatory Authorities, consideration must be given, and a plan developed as to how to communicate this regulatory action to all remaining Regulatory Authorities.

### 4.1.8 Follow-up Activities

Should there be any changes to information previously submitted, or results of follow-up actions that impact on the understanding of a reported INSI, due consideration should be given to the necessity of sending an update to the relevant internal stakeholders and where appropriate, action must be taken to submit this relevant data to Regulatory Authorities, Investigators and Ethics Committees as appropriate.

Follow-up activities are likely to include, but are not limited to, actions resulting from Safety Information Review Committee (SIRC) such as label updates or issuance of a DHPC letter, consideration of an update to the Risk Management Plan (RMP) and inclusion in the next Periodic Benefit Risk Evaluation Report (PBRER).

#### 4.1.9 Notification to Business Partners

When a potential INSI has been identified, SSaMT must determine the need to consult with Business Partners regarding the INSI in line with the requirements of the pharmacovigilance Agreement. These notifications and discussions are documented within the SSaMT minutes, see Section 4.6.

## 4.2 Process for Identification of Important New Safety Issues from Non-clinical Sources

| Number | Responsibility | Activity |
|---|---|---|
| **4.2.1** | Any AZ personnel or representatives acting on behalf of AZ | Communicates a potential non-clinical INSI immediately (within 24 hours) for evaluation to an appropriately qualified individual, for example the TPL. |
| **4.2.2** | TPL | Makes an initial assessment of the new information to determine if it is a possible INSI (see 4.1.2.1).<br>Liaises with the SSaMT and appropriate technical experts, internal or external, and ensures that any data, that is relevant and available within the timeframe, is collated to enable the endorsement process to proceed for the potential INSI. |
| **4.2.3** | TPL | In conjunction with the SSaMT, immediately escalates the information within their functional line management and to Safety Assessment Review Board (SARB) to request a review. |
| **4.2.4** | GSP | Immediately notifies the SPVMD, PV Science Director, PS TA VP and QPPV (for EEA submitted/authorized products only). |
| **4.2.5** | SARB | Holds SARB meeting as soon as possible to which key project team members are invited, for example the SPVMD, SSaMTL, Global Regulatory Lead (GRL), GSL, Global Medical Lead (GML) (if molecule is under clinical study) and the QPPV (for EEA submitted/authorized products only).<br>Reviews the finding and makes a recommendation as to whether an INSI exists and warrants expedited notification.<br>If warranted, makes recommendation to stop dosing in clinical studies (see Section 4.4 and 4.5). |
| **4.2.6** | TPL | **If the recommendation for expedited reporting is no:**<br>Communicates decision to SPVMD, SSaMTL and relevant project team members including GSL and GML if applicable.<br>This recommendation is endorsed by the SPVMD and |

| Number | Responsibility | Activity |
|---|---|---|
| | | SSaMT if in agreement with SARB recommendation.<br>The decision is appropriately documented (refer to Section 4.6) and no further endorsement or action within the scope of this SOP is required.<br>If this recommendation is not endorsed by the SPVMD and SSaMT proceed to 4.2.7. |
| **4.2.7** | TPL or SPVMD | **If recommendation for expedited reporting is yes:**<br>TPL informs the GSP and SPVMD who then take the recommendation to the PS TA VP and EU QPPV for endorsement.<br>**If recommendation to not expedite reporting is rejected by the SPVMD and SSaMT:** The SPVMD informs SARB, TPL, GSL/GML (if applicable) and takes the INSI to the PS TA VP and EU QPPV for endorsement. |
| **4.2.8** | PS TA VP, QPPV | Provides final endorsement regarding whether the recommendation for expedited reporting is upheld.<br>Where an INSI impacts a product submitted/authorized in the EEA, the EU QPPV will jointly endorse the recommendation with the PS TA VP.<br>Clock-start date for submission will be the date of endorsement (see Section 4.1.5). Contacts the Clinical Development TA VP if USMs are expected for clinical programs. |
| **4.2.9** | SPVMD | Informs SARB, SSaMT, TPL, GRL, GSL and appropriate project team members of endorsement decision.<br>If the decision is to expedite reporting informs the Executive Safety Board (ESB) as well.<br>The decision is documented according to Section 4.6. |
| **4.2.10** | SSaMT | Recommends whether the issue should be taken forward to SIRC and suggests a timeline. If yes, notifies the SIRC Chairperson and follows procedure per SOP: 8-P124-CV-X Safety Information Review Committee (SIRC). |
| **4.2.11** | SSaMT | Proposes other required actions such as protocol amendments for changes in study design which will be escalated through the CDL and GML for endorsement by the appropriate Clinical Development TA VP and for ensuring required actions are completed. See USM SSaMT Evaluation Form in AZ Doc. |
| **4.2.12** | Clinical Development TA VP | In consultation with the PS TA VP, endorses other required actions such as protocol amendments, USM (Section 4.5), suspension of dosing. |

## 4.3 Process for Identification of Important New Safety Issues from all other sources

| Number | Responsibility | Activity |
|---|---|---|
| **4.3.1** | Any AZ personnel, or representatives acting on behalf of AZ | Communicates any potential INSI immediately (within 24 hours) for evaluation to an appropriately qualified individual (for example, GSP, SPVMD, any SSaMT member, Clinical Medical Monitor (CMM)/Study Team Physician (STP), other PS representative who will immediately inform the SSaMTL. |
| **4.3.2** | SSaMTL | Ensures that any data that is relevant and available within the timeframe is collated to enable assessment of the potential INSI.<br>Makes arrangement for key experts to discuss and evaluate any potential INSI. The experts should include the SPVMD, SSaMT (GSL/CDL/GRL), and other technical experts as needed (for example Clinical Study Physician (CSP) or statistician). These discussions must be prioritised. |
| **4.3.3** | GSP | Based on the available data, in consultation with the SSaMT and SPVMD makes a recommendation whether the new information represents an INSI and warrants expedited notification. |
| **4.3.4** | SSaMTL | **If the recommendation for expedited reporting is no:**<br>Communicates this to appropriate project team members and documents the decision per Section 4.6. No further endorsement or action within the scope of this SOP is required.<br>The routine signal detection process will be followed (Refer to SOP: Routine Detection and Management of Safety Signals). |
| **4.3.5** | SPVMD | **If the recommendation for expedited reporting is yes:**<br>Takes the recommendation to the PS TA VP and QPPV (for EEA submitted/authorized products only), for endorsement (see Section 4.1.2.2). |
| **4.3.6** | PS TA VP, QPPV | PS TA VP: Provides final endorsement regarding whether the recommendation for expedited reporting is upheld.<br>Where an INSI impacts products submitted/authorized in the EEA, the QPPV will jointly endorse the recommendation with the PS TA VP.<br>If recommendation for expedited notification is not upheld informs SPVMD, GSP and SSaMTL see 4.3.4.<br>If recommendation for expedited notification is upheld proceed to 4.3.7.<br>Clock-start date for submission will be the date of this |

| Number | Responsibility | Activity |
|---|---|---|
| | | endorsement (see Section 4.1.5). Contacts the Clinical Development TA VP if USMs are expected for clinical programs. |
| **4.3.7** | SPVMD | Informs the ESB and communicates the endorsement decision to SSaMT, GRL/GSL and any other relevant stakeholders.<br>Documents the endorsement decision according to Section 4.6. |
| **4.3.8** | SSaMT | Determines whether the issue should be taken forward to SIRC. If yes, notifies the SIRC Chairperson and follows procedure per SOP: SIRC.<br>Proposes other required actions such as protocol amendments for changes in study design which will be escalated through the CDL for endorsement by the appropriate Clinical Development TA VP and for ensuring required actions are completed. See USM SSaMT Evaluation Form. |
| **4.3.9** | Clinical Development TA VP | In consultation with the PS TA VP, endorses other required actions such as protocol amendments, USM or suspension of dosing. |

## 4.4 Process for Preparation of Documents and Reporting

| Number | Responsibility | Activity |
|---|---|---|
| **4.4.1** | SSaMTL, GSL | Authors documents for submission with input from SSaMT, SPVMD, GRL, CDL, TPL and any other relevant experts and provides to the Regulatory Project Manager (RPM).<br>For reporting INSIs to clinical sites requiring USMs see the USM Template.<br>Unless otherwise specified by local regulatory reporting requirements, the global documentation should be submitted in English and, if necessary, a cover letter in local language can be prepared by the local MC.<br>For products submitted/authorized in the EEA, the QPPV will review and endorse the communication for submission to EMA. |
| **4.4.2** | CDL,<br>Global RPM | Identify the specific reporting requirements (for example types of report required and to whom), considering the factors listed in Section 4.1.7. |
| | | **'Unanticipated Problems' per FDA Guidance** |

| Number | Responsibility | Activity |
|---|---|---|
| **4.4.3** | GSP, CDL | Provides a report of the 'Unanticipated Problem' to the General Issues Mailbox for prompt distribution:<br>dsewilm-generalissues@astrazeneca.com |
| | | **Submit to Regulatory Authorities** |
| **4.4.4** | RPM and EU QPPV | Ensures timely global submission of expedited notifications to relevant Regulatory Authorities according to the timelines described in Section 4.1.6.<br>All submissions are tracked see section 4.1.7.<br>Notifies QPPV and MC Regulatory Affairs (MC RA) of the expedited notification and copies the "MC Support Mailbox" so that the relevant MC PS Managers (PSMs) are appropriately informed. |
| **4.4.5** | MC RA | Submits the notification to local Regulatory Authorities within locally defined requirements and timelines.<br>All submissions are tracked see section 4.1.7. |
| **4.4.6** | QPPV or Delegate | For EEA submitted/authorized products, submits Emerging Safety Issue notification to EMA (see Section 4.1.6.1). |
| **4.4.7** | MC RA | Informs RPM of the date when submission to local Regulatory Authority has been made and stores submission documents in ANGEL DM (according to document specific processes). |
| **4.4.8** | RPM | Informs SSaMTL and GRL of submission dates so that they can be documented.<br>The RPM ensures that submission dates plus any communications in ANGEL DM resulting from the notification are stored in ANGEL Register. |
| **4.4.9** | SSaMTL, CDL | Documents that INSI have been notified within the defined timelines (see Section 4.6). |
| | | **Submit to Ethics Committees and Investigators** |
| **4.4.10** | CDL | Ensures timely global submission of expedited notifications and USMs (see section 4.5) to Ethics Committees and Investigators through the Study Managers according to the timelines described in Section 4.1.6.<br>Notifies MC Patient Safety Manager (PSM) of the expedited notification and copies to the "MC Support Mailbox" so that the relevant MC PSMs are appropriately |

| Number | Responsibility | Activity |
|---|---|---|
| | | informed. |
| **4.4.11** | Study Manager | Ensures all Local Study Managers and/or CRO on impacted studies (including internally managed, outsourced studies, ESRs and platform studies) are notified of the INSI and any USM, see Investigator Acknowledgement of USM form and Urgent Safety Measure Operational Checklist. |
| **4.4.12** | Local Study Manager/CRO | Ensures submission of the expedited notifications to the relevant Ethics Committees and Investigators within their territory and according to locally defined requirements and timelines.<br>Informs Study Manager of the date when submission to Ethics Committees and Investigators has been made.<br>Study Manager informs CDL and when submission to Ethics Committees and Investigators is completed. |
| **4.4.13** | CDL | Informs SSaMTL of the submissions and dates. |
| **4.4.14** | SSaMTL | Documents that INSI have been notified within the defined timelines (see Section 4.6). |
| | | **Notification to Global Safety and Health** |
| **4.4.15** | SSaMTL | Notifies Global Safety and Health (Global SHE) of an INSI so it can be considered for relevance to occupational toxicology. The notification should be sent to OccTox@astrazeneca.com. |
| | | **Other Actions** |
| | GSL | Ensures that an INSI is appropriately incorporated into the benefit-risk assessment for the product within IB and other applicable study documents. |
| | RPM | Notifies other markets with previous study participants as appropriate as per section 4.1.7. |
| | SSaMTL | In the event of changes to information previously submitted, coordinates any subsequent communications and follow-up activities relating to the INSI (see Section 4.1.8). |

## 4.5 Post USM Notification for Clinical Studies

| Number | Responsibility | Activity |
|---|---|---|
| **4.5.1** | Study Manager | Works with MC RA representative to ensure any substantial amendment (i.e. any changes to the CTA documents such as protocol and ICF) are made within local requirement timelines. Any potential translations required must be factored into the Regulatory Authority submission timelines. |
| **4.5.2** | Study Manager | Reviews and updates PharmaCM for any changes needed due to the USM. |
| **4.5.3** | MC RA | Send any Authority questions either from the initial USM submission or from the Substantial Amendment submission to the GRL/Study RPM. The GRL then distributes to the appropriate stakeholders. |
| **4.5.4** | Study RPM | Co-ordinates responses back to the MC RAs. |
| **4.5.5** | Global Regulatory Operations, CDL | Global Regulatory Ops: Ensures all documentation around the USM and the Substantial Amendments are documented in ANGEL DM (according to document specific processes).<br>CDL: Ensures all documentation for USMs are archived in the eTMF. |

## 4.6 Documentation

| Number | Responsibility | Activity |
|---|---|---|
| **4.6.1** | SSaMTL | The SSaMTL will ensure the following is documented:<br>• Potential non-clinical INSI, and the endorsement decision by the PS TA VP (see Section 4.2): typically documented in the SARB meeting minutes.<br>• A potential INSI from other sources that is escalated for evaluation (see Section 4.3). In the event that a potential INSI is judged *not* to constitute an INSI (either at the recommendation or endorsement step in the process) this is recorded with a brief description of the reasons why not in the SSaMT minutes.<br>• All endorsed INSI (non-clinical and clinical) requiring notification to Regulatory Authorities, Ethics |

| Number | Responsibility | Activity |
|---|---|---|
| | | Committees or Investigators: the Regulatory Clock Start date (see Section 4.1.5) and confirmation that INSI were notified within the defined timelines must be recorded in the SSaMT minutes. The clinical reasoning for not submitting an expedited notification to certain groups of Investigators and Ethics Committees where more than one indication exists for a product must be recorded in the SSaMT minutes as well. |

# 5. ROLES AND RESPONSIBILITIES

Responsible parties in the procedure may appropriately delegate an activity but retain responsibility for the outcome. Activities delegated to external contractors (for example CROs or vendor partners) must be documented in a contract. Detailed responsibilities for each task or step of the procedure are provided in Section 4. A detailed RACI for responsibilities is provided in Appendix A.

The MC RA responsibilities may be outsourced to a CRO for studies being conducted via preferred providers, however, the GRL and Study RPM should always be copied into any communication and be the contact point for the vendor. In addition, the vendor should ensure all relevant documentation is sent to GRO.

# 6. GLOSSARY

Any terms listed in this section are defined by the author as only used within the context of this Procedural Document. Terms commonly used across this and other Procedural Documents can be found in glossary with the exception of following below:

| Term | Definition |
|---|---|
| Safety Signal | Information arising from one or multiple sources (including observations and experiments) which suggests a new potentially causal association, or a new aspect of a known association between an intervention and an event or set of related events that is judged to be of sufficient likelihood to justify verificatory action. |
| Urgent Safety Measure | A procedure or action which is not defined in the protocol that can be put in place with immediate effect prior to obtaining authorisation by an Agency or Ethics Committee in order to protect clinical trial participants from any immediate hazard to their health and safety. |

| Term | Definition |
|---|---|
| Clinical Delivery Lead | This is a generic role description to include the roles of Director Clinical Development (DCD) or Clinical Program Team Lead (CPTL) for or Director Scientific Project Management (DSPM). |
| Global Science Leader | This is a generic role description to include the roles of Global Clinical Leader (GCL) or Medical Science Director (MSD) |
| Study Manager | This is a generic role description to include the roles of Global Study Leader or Associate Director Study Management (ADSM) |

# 7. REFERENCES

This procedure follows applicable local and/or regional regulations, ICH Good Clinical Practice, and Good Pharmacovigilance Practice.

- Standard: 7-S4-CV-X -The European Qualified Person Responsible for Pharmacovigilance AZDoc007257
- Standard: 7-S3-CV-X- Oversight of Patient Safety Clinical Products AZDoc007113
- SOP: Medical Review of Individual Case Safety Reports within the Case Handling Process AZDoc007154
- SOP: Expedited Reporting of Individual Case Safety Reports to Regulatory Authorities AZDoc007158
- SOP: 7-P35-Cv-X - Routine Detection and Management of Safety Signals AZDoc007184
- SOP: 8-P124-CV-X Safety Information Review Committee (SIRC) AZDoc007241
- SOP Direct Health Care Professional Communication for DHPC AZDoc0010091
- Urgent Safety Memo Template
- Investigator Acknowledgement of USM Form
- USM SSaMT Evaluation Form
- Urgent Safety Measure Operational Checklist

# 8. REVISION HISTORY

| Version | Description of Change |
|---|---|
| 1.0 | New document. |
| 2.0 | Updated to reflect organizational changes, clarification of terminology, inclusion of language from retired SOP “Safety Issue Management”, additional detail on |

| Version | Description of Change |
|---|---|
| | regulatory process for submission and removal of appendix B. |
| 3.0 | Minor update of references. Section 4.1.1.3 Removal of reference to retired SOP “Safety Issue Management”. |

# 9. APPENDICES

**Appendix A**: RACI Table.

## **Appendix A**: RACI Table

| | SSaMTL | TPL | SPVMD | GSP | GRL | RPM | GSL | SARB | CDL | PS TA VP | EU QPPV | ESB | Study Manager | Local Study Manager | MC Reg |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| Escalation of decision to recommend a notification of Important New Safety Issue – non-clinical | R | R | | | | | | A | | | | | | | |
| Endorsement of recommendation; non-clinical | | | R | | | | | | | A | A* | I | | | |
| Escalation of decision to recommend a notification of Important New Safety Issue - clinical | | | R | A | | | | | | | | | | | |
| Endorsement of recommendation; clinical | | | | | | | | | | A | A* | I | | | |
| Dispute resolution | R | R | R | | | | | R | | A | R* | A | | | |
| Maintain log of Important New Safety Issues & outcome of notification | A | | | | R | R | | | R | | | | R | | |
| Author document for submission | A | R | R | R | R | | A | | R | | | | | | |
| Document FDA 'Unanticipated Problems' | | | | A | | | | | R | | | | R | | |
| Consideration of scope of submission | R | | | | A | R | A | | | | | | | | R |
| Submission of notifications to Regulatory Authorities & inform team | | | | | A | R | | | R | | R* | | | | R |
| Submission of notifications to ECs and Investigators & inform team | | | | | | R | | | A | | | | R | R | |
| Notify MCs from past studies (if appliciable) | A | | | | | R | | | R | | | | | R | |
| Inform Global SHE | A | | | | | | | | | | | | | | |
| Coordination of follow up activities | A | | | | | | | | | | | | | | |
| Follow up actions relating to SIRC | A | | | | | | | | | | | | | | |

*For products submitted/authorised within the EEA